Medbury Healthcare Group

The conversion clinic

Alameda's Kenya plan, rebuilt for Abuja on Nigerian costs

Medbury Healthcare Group / 49 per cent shareholder / Private and confidential

Ground floor, Lyfe Place Abuja / 113.0 sqm, five consulting rooms

Not a clinic that refers. A clinic whose product is the referral.

Alameda runs this model in Kenya already, and the deck they gave us is the same product in a different market. The consultation is not the business. The consultation is how you find the patient who needs an admission at Al Salam, and the admission is where the money is. Three revenue streams, and the third is four times the first.

Revenue stream	Y1	Y5	What it actually is
OP consultation	152	289	A fee for seeing the patient. Local consultants at NGN 40,000, Al Salam consultants flying in at NGN 85,000
Diagnostic referral	28	57	20% commission on the pre travel workup, ordered from Medbury Diagnostics in the chalet. Money that already stays inside the group
Inpatient referral	142	394	15% of what the patient spends at Al Salam. This is the business
Total revenue, NGN M	**322**	**740**	

Everything turns on how many consultations become an admission.

Alameda's Kenya plan books referral revenue of USD 204,120 in year one. At a 15 per cent commission on a USD 6,000 discharge, that is 227 admissions against 5,544 consultations. A conversion of 4.09 per cent. It is the only number in their deck that is an observation rather than an assumption, and it is the number this whole case rests on.

4.09%
KENYA, OBSERVED
What Alameda's own plan implies

2.0%
ABUJA YEAR ONE
We open below them and earn our way up

2.0%
ABUJA YEAR FIVE
Still inside what they have already done

283
ADMISSIONS A YEAR
About 24 a month travelling to Al Salam

At half Kenya's conversion this is still a business: 37 per cent IRR and capital back in 2.6 years. With no commission at all it is not a business, it is a loss. The consultation and diagnostic lines do not cover the cost base, so the referral is not the upside on this clinic. It is the clinic. Ask Alameda for the cohort behind the 4.09 per cent before anyone signs: referred, travelled, paid.

Five rooms, but a conversion clinic is not a polyclinic.

	Y1	Y2	Y3	Y4	Y5
Consulting rooms in use	1.5	2.0	2.5	2.5	2.5
Consultations a room day	8	9	9	10	10
Local consultations	3,024	4,536	5,670	6,300	6,300
Al Salam consultants, 3 days a month	360	396	432	432	432
Total consultations	3,384	4,932	6,102	6,732	6,732
Admissions converted	**102**	**173**	**244**	**283**	**283**

Kenya runs two clinics to reach 5,544 consultations. We reach 6,732 from one, using half the rooms we are renting. The other two and a half rooms are headroom, not a forecast: if the conversion holds, they are the cheapest growth available, and if it does not, we have not built a cost base around them.

Same product, two thirds of the cost base.

Year one	Kenya, NGN M	Abuja, NGN M	Why it differs
Revenue	598	322	Kenya is two clinics. We are one, at Nigerian consultation prices
Occupancy	45	53	One all in charge from the landlord. No separate rent, parking, service charge or utilities, because Lyfe Place already includes them
Personnel	56	30	The same four posts. Nigerian salaries plus the 22 per cent employer stack
Capital	236	102	Kenya buys its own fit-out. Ours is the landlord's and comes back through the rent, so we fund category B only
EBITDA margin, Y1	25%	14%	We open thinner because we ramp the conversion. By Y3 we are ahead

The whole difference is cost, not cleverness. Nigerian salaries are a third of the dollar cost Kenya carries, and the landlord funds the fit-out. That is why this returns more than Alameda's own plan on the same conversion rate, and it is the honest answer if they ask why our numbers look better than theirs.

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Five years, NGN millions.

	Y1	Y2	Y3	Y4	Y5
OP consultation	152	215	264	289	289
Diagnostic referral	28	41	51	57	57
Inpatient referral	142	241	340	394	394
Total revenue	322	497	655	740	740
Doctor fees and referral share	88	126	154	170	170
Gross margin	219	349	471	536	536
Occupancy	53	59	67	75	84
Personnel	30	30	30	30	30
Operating cost, all in	173	182	199	217	236
EBITDA	46	166	272	319	299
Depreciation	15	15	15	15	15
Net income	21	106	180	213	199
Free cash flow	37	121	195	228	214
EBITDA margin	**14%**	**33%**	**42%**	**43%**	**40%**

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Ten ways this could go.

Scenario	Y1 EBITDA	Y5 EBITDA	Y5 margin	IRR	NPV	Payback
Base case	46	299	40%	94%	268	1.5 yr
Kenya parity from year one	95	290	40%	113%	311	1.2 yr
Conversion half of Kenya	1	103	19%	37%	37	2.6 yr
Commission cut to 10 per cent	1	175	29%	54%	103	2.3 yr
No referral line at all	-89	-75	-22%	n/a	-286	never
Naira strengthens to 1,240	19	225	34%	71%	171	1.9 yr
Naira weakens to 2,015	86	412	48%	125%	415	1.2 yr
Volume 25 per cent below plan	-4	181	31%	53%	103	2.3 yr
Upside: 3 rooms, 5% conversion	138	483	50%	157%	539	1.0 yr
Severe: half conversion and 25% less volume	-38	29	7%	-6%	-82	never

The commission is not the upside. It is the business.

It survives being wrong about the conversion.

At 2.0 per cent, half what Kenya has already done, it still returns 37 per cent and returns the capital in 2.6 years. Cutting the commission to 10 per cent returns 54 per cent. Neither of those is a reason not to do it.

It does not survive losing the referral.

With no commission the clinic loses NGN 75 M a year by year five on NGN 345 M of revenue. The consultation gives 60 per cent away to the consultant and the cost base is NGN 236 M. There is no version of this that works as an outpatient clinic.

Two things wrong at once is the real risk.

Half the conversion on its own is survivable. Half the conversion with 25 per cent less volume is a -6 per cent IRR and the capital never comes back. The downsides are not independent: thin volume and weak conversion arrive together, because both mean the proposition is not landing.

The naira cuts both ways and nobody controls it.

The referral line is dollar linked and every cost is naira. At 1,240 the IRR is 71 per cent, at 2,015 it is 125 per cent. That spread is wider than most of the operating assumptions in this deck and none of it is management skill.

Capital back inside two years.

NGN 102 M
CAPITAL
Category B only. The landlord funds the fit-out

94%
IRR
Five years, no terminal value

NGN 268 M
NPV AT 25%
Naira nominal hurdle

1.5 yrs
PAYBACK
From opening, not from instruction

	Y1	Y2	Y3	Y4	Y5
Free cash flow	37	121	195	228	214
Net income	21	106	180	213	199
To Medbury at 49%	**10**	**52**	**88**	**104**	**97**

Medbury's 49 per cent of net income is NGN 352 M over five years, against a 49 per cent share of the NGN 102 M capital, which is NGN 50 M in. The clinic also pays us NGN 53 M of rent in year one as its landlord, rising with the lease. Both sides of that are ours and they should be read together.

Every driver, and where it came from.

Driver	Value	Source
Commission from Al Salam	15%	Confirmed with Alameda. The single largest line in the model
Average revenue per discharge	USD 6,000	Alameda Kenya plan
Conversion to admission	2.0% to 2.0%	Kenya implies 4.09%. We open below it. UNVERIFIED, and the one to test
Local consultation	NGN 40,000	Medipark consultant survey, n=20: 26%% charge 25 to 50k
Al Salam consultation	NGN 85,000	Premium for a visiting international consultant
Pre travel workup	35% of patients, NGN 120,000	Bloods, imaging, cardiac. Commission 20% from Medbury Diagnostics
Doctor fee	60% local, 50% foreign	Alameda Kenya plan, unchanged
Occupancy	NGN 53.1 M, +12% a year	113.0 sqm at NGN 470,000 all in, from the Lyfe Place rate card
Employer on-costs	22%	Pension, 13th month, NSITF, ITF, group life, HMO
FX	USD/NGN 1,550	Reference rate across the pack

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One assumption carries the case, and it is not ours.

Risk	Impact	What to do about it
The conversion rate is lower than Kenya's	Severe	Ask for the Kenya cohort now: referred, travelled, paid. Structure the first year so the clinic covers its costs on consultation and diagnostics alone
Patients are referred but do not travel	Severe	Visa, cost and family friction all sit between the referral and the admission. Commission is earned on arrival, so this is pure leakage
Al Salam does not pay on time	High	The commission is a receivable on a foreign hospital. Net 30 in the agreement, with a reconciliation the clinic can audit
Naira strengthens	Medium	The referral line is dollar linked and the cost base is naira. A stronger naira cuts revenue and not costs. A weaker one is a tailwind
Medical tourism attracts scrutiny	Medium	Referring patients abroad for money is a reputational question in a market that wants capacity built at home. The diagnostics and consultation lines are the defensible part of the story

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Three things before anyone signs.

	What	Why
1	The Kenya conversion cohort	How many of the 5,544 consultations were referred, how many travelled, how many paid. The 4.09 per cent is the case and we are taking it on trust
2	The commission agreement with Al Salam, in writing	15 per cent of what, measured how, paid when, audited by whom. A commission on a foreign hospital's billing is only as good as the reconciliation behind it
3	Who funds the NGN 102 M of category B	Medbury at 49 per cent is NGN 50 M. The lease and the shareholding are two different conversations and they should not be settled in one meeting

On the numbers as they stand this is a good business: NGN 102 M of capital, 94 per cent IRR, capital back in 1.5 years, and NGN 352 M of net income to Medbury over five years on top of the NGN 53 M a year it pays us in rent. But it is one assumption deep, and that assumption is Alameda's, not ours.